

RIVERSIDE SURGICAL CENTER
Operative Report – Ambulatory Surgery

Patient: Soto, Evelyn
DOB: 1972-11-09 MRN: RSC-2024-0820
Date of Surgery: 05/20/2024
Surgeon: Marcus Lavoie, MD (NPI 1665544331)
Assistant: Tara Ruiz, PA-C
Anesthesia: MAC + local infiltration (separate billing)

PREOPERATIVE DIAGNOSIS

Symptomatic incisional hernia, midline abdomen, status post prior open appendectomy (2009). ICD-10: K43.2.

POSTOPERATIVE DIAGNOSIS

Same.

PROCEDURE PERFORMED

CPT 49560 – Initial repair of incisional or ventral hernia, reducible (open approach).

INDICATION FOR SURGERY

51-year-old female with a 4 cm reducible incisional hernia at the inferior pole of her appendectomy scar, progressively enlarging over 14 months. Patient reports pulling discomfort with prolonged standing and a recent episode of incarceration that reduced spontaneously (documented in ED visit 04/02/2024). Conservative management with abdominal binder ineffective. Risk of progression to strangulation discussed; patient elected for elective repair.

PRIOR-PAYER COORDINATION

Patient has primary coverage under Spouse's BCBS PP0; this submission to Anthem is the secondary claim. Anthem received the primary EOB (BCBS payment \$2,500 against \$4,650 allowed; patient deductible satisfied; primary patient responsibility \$0). Anthem's adjudication line OA-23 reflects coordination of benefits; appeal requests review for any remaining Anthem-side allowable amount.

OPERATIVE DETAILS (abbreviated)

- General positioning, supine. Time-out completed.
- Standard prep and drape. Midline scar excised. Hernia sac identified and dissected free of subcutaneous fat. Sac contents (omentum) reduced into abdomen.
- Fascial defect measured 4 cm. Repaired primarily with interrupted 0-Prolene Mayo-style overlap. Reinforced with 6x6 cm onlay polypropylene mesh secured with #2-0 Prolene interrupted sutures.
- Hemostasis confirmed. Subcutaneous tissue approximated with 3-0 Vicryl. Skin closed with 4-0 Monocryl subcuticular.
- Sponge, needle, and instrument counts correct.
- Patient transported to PACU in stable condition.

COMPLICATIONS

None.

DISPOSITION

Discharge same day, post-PACU monitoring per protocol. Follow-up office visit 14 days. Pain management plan provided.

Electronically signed: Lavoie, Marcus MD on 05/20/2024 11:47